

SURGICAL PATHOLOGY

Case Number :

Diagnosis:

A: Ureter, right margin, biopsy
- No tumor seen

B: Ureter, left margin, biopsy
- No tumor seen

C: Bladder and prostate, radical cystoprostatectomy

Procedure: radical cystoprostatectomy

Bladder:

Tumor histologic type: urothelial carcinoma

Tumor histologic grade (WHO classification): high grade

Microscopic tumor size (greatest dimension): 4 cm

Microscopic extent of invasion: Tumor invades through muscularis propria into perivesical adipose tissue, extensively.

Tumor site/focality of tumor: unifocal, posterior wall, left lateral wall, trigone (one large contiguous tumor)

Angiolymphatic space invasion: present, extensive

Associated epithelial lesions: not identified

Ureters: No tumor seen, bilateral

Histologic assessment of surgical margins:

Ureter, left: free of tumor

Ureter, right: free of tumor

Urethra: free of tumor

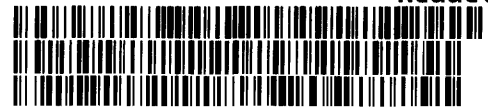
Paravesicular soft tissue: closest margin is 1.5 mm from this margin

Other significant findings: ulceration is present overlying the invasive cancer

ICD-O-3
Carcinoma, urothelial NOS
Site Bladder NOS 8/20/13
path C67.9
Overlapping lesion of
bladder C67.8
JW 8/19/13

UUID: 8D56AE41-2A0C-4D82-971E-989CB5762FFF
TCGA-CU-A72E-01A-PR

Redacted



Lymph nodes: Metastatic urothelial carcinoma is present in a total of 6 of 41 lymph nodes. This lymph node count includes 3 of 3 positive lymph nodes from perivesicular nodes, and 2 positive nodes from the left pelvic region, and 1 positive node from the right pelvic region.

Size and location of largest lymph node metastasis: 2.5 cm, left perivesicular lymph node

Extracapsular extension: present, extensive

Diagnoses of other organs or structures: See prostate template below.

AJCC Pathologic TNM Stage: pT3b pN2

NOTE: This pathologic stage assessment is based on information available at the time of this report, and is subject to change pending clinical review and additional information.

Prostate:

Tumor histologic type: adenocarcinoma

Tumor grade (Gleason system): 6 (3+3)

Approximate percentage of prostate involved by tumor: less than 5%

Extent of tumor:

Location of tumor involvement in prostate: left apex

Confinement/non-confinement of tumor within the prostatic capsule: tumor confined to prostate capsule

Lymphovascular space invasion: not identified

Seminal vesicles (invasion of muscular wall required): No tumor seen, bilateral

Histologic assessment of surgical margins: Surgical margins free of prostate carcinoma.

Other significant findings: Benign prostatic hyperplasia.

Lymph nodes: All lymph nodes are negative for prostate carcinoma.

AJCC Staging:

pT2A

pN0 (for prostate cancer)

This staging information is based on this pathologic specimen and may be incomplete. A comprehensive review of all available information is recommended to determine final staging.

D: Lymph node, left pelvic, regional resection

- Metastatic urothelial carcinoma present in two of seventeen lymph nodes

E: Lymph node, right pelvic, regional resection

- Metastatic urothelial carcinoma in one of twenty-one lymph nodes

Intraoperative Consult Diagnosis:

A frozen section consultation was requested by Dr. in OR #

Specimen delivery time is (A) and (B).

FSA1: Ureter, right margin, biopsy

- No carcinoma identified

FSB1: Ureter, left margin, biopsy

- No tumor seen

Drs. on at (A)

Drs. on at (B)

Frozen Section Pathologist:

Clinical History:

-year-old male with muscle invasive urothelial carcinoma, high grade, involving lamina propria and muscularis propria.

Gross Description:

Received are five appropriately labeled containers. Specimens A and B are received fresh for frozen section.

Container A is additionally labeled "right ureteral margin for frozen section." It consists of a 3 mm length x 3 mm circumference white/tan tubular segment with attached fat. The specimen is entirely submitted for frozen section and resubmitted in block FSA1; fat remains in the container.

Container B is additionally labeled "left ureter margin for frozen section." It consists of a 4 mm in length x 5 mm in

diameter white/tan tissue fragment with a partial lumen and attached fatty tissue. The specimen is entirely submitted for frozen section and resubmitted in block FSB1. The remaining fat is submitted for permanent section in block B2.

Container C is additionally labeled "bladder and prostate."

Specimen fixation: formalin

Type of specimen(s) received: radical cystoprostatectomy

Size of overall specimen: 25 cm x 14 cm x 7 cm

Size of urinary bladder: 7 x 6 x 5 cm

Size of additional organs: Prostate: 4.5 x 3.5 x 3.0 cm. Seminal vesicles are present, size cannot be determined.

Orientation: Inking: right perivesicular=blue, left perivesicular=black, distal urethral margin=yellow

Tumor size: 4 x 3.5 x 2.5 cm

Tumor description: The mucosal surface of the tumor is ulcerated and erythematous, the cut surface is tan/white, nodular, firm and ill-defined.

Tumor location: posterior and left lateral bladder, extending from the trigone to 4 cm superior to the ureteral orifice

Distance of tumor to margins: Distal urethral margin, 4 cm; perivesicular fat, 0.3 cm; left ureter margin, 1.4 cm; right ureter margin, 3.5 cm

Other mucosal lesions: none

Description of other organs or structures: Attached to the left lateral perivesicular fat is a 5.5 x 2.5 x 1.5 cm lymph node. In addition, adjacent to this there is a 2.5 x 1.2 x 1.0 cm lymph node candidate. The cut surface reveals that it is grossly positive for Tumor. The prostate has an unremarkable cut surface with mild amount of cystic change and a few small nodules.

Digital photograph taken: not taken

Tissue submitted for special investigation: yes, to

Block Summary: (Inking: right perivesicular=blue, left perivesicular=black, distal urethral=yellow)

C1 - right ureter margin
C2 - left ureter margin
C3-C5 - distal urethral margin, radial sections
C6 - left ureteral orifice
C7 - right ureteral orifice
C8 - trigone, perpendicular
C9,C10 - one lymph node candidate attached to the left perivesicular fat, grossly positive
C11 - one lymph node candidate
C12-C25 largest lymph node candidate attached to the left perivesicular fat, serially sectioned
C26 - additional indurated fat adjacent to the largest lymph node
C27 - tumor closest to the black inked perivesicular fat margin, posterior bladder
C28 - trigone, and attached fat
C29 - tumor at the left lateral bladder wall
C30 - normal right side of bladder
C31 - left prostate apex
C32 - left posterior mid prostate
C33,C34 - left prostate base, left seminal vesicle in C34
C35 - right prostate apex
C36 - right mid prostate, posterior
C37 - right prostate base, with right seminal vesicle
Tissue remains in formalin.

Container D is additionally labeled "left pelvic lymph node." It consists of a 5 x 5 x 1.5 cm aggregate of adipose tissue and lymph node candidates. Numerous lymph node candidates are identified, ranging from 0.2 to 3.0 cm in greatest dimension.

Block summary:

D1,D2 - one lymph node, serially sectioned
D3,D4 - one lymph node, serially sectioned
D5 - one lymph node
D6 - one lymph node, bisected
D7 - three lymph nodes
D8 - one lymph node, bisected
D9 - three lymph nodes
D10 - one lymph node, bisected
D11 - three lymph nodes
D12 - three lymph nodes

Container E is additionally labeled "right pelvic lymph node."
 It consists of a 6 x 6 x 3 cm aggregate of adipose tissue and lymph node candidates. Numerous lymph node candidates are identified ranging from 0.2 to 3.0 cm in greatest dimension.

Block summary:

E1-E5 - one lymph node, serially sectioned
 E6-E8 - one lymph node, serially sectioned
 E9-E12 - one lymph node, serially sectioned
 E13-E14 - one lymph node
 E15-E16 - one lymph node
 E17 - one lymph node
 E18-E19 - one lymph node, serially sectioned
 E20 - one lymph node
 E21 - one lymph node
 E22-E25 - one lymph node per cassette
 E26 - four lymph nodes
 E27 - four lymph nodes

Cutaneous Merkel Cell carcinoma,
 Anterior orbital Fossa - rec'd
 locoregional radiation. BCP
 Patient did receive BCG > 90 days prior to resect tumor procedure

W 7/30/13

Criteria	Yes	No
Diagnosis Discrepancy		☒
Primary Tumor Site Discrepancy		☒
HPAA Discrepancy		☒
Prior Malignancy History	☒	
Qual/Synchronous Primary Tumor	☒	
Case is (circle)	QUALIFIED	DISQUALIFIED
Reviewed by	10/11/13	